



Dysphagia Screening and Assessment Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	Allied Health, Nursing

2. Purpose

This procedure establishes minimum practice standards for performing dysphagia screening by:

- Identifying patients with swallowing difficulties, or who are *at risk* of swallowing difficulties and/or aspiration.
- Preventing unnecessary nil by mouth (NBM) status, by using a screening tool that is based on a risk management model.

3. Equipment

- Royal Brisbane Women's Hospital (RBWH) Dysphagia Screening Tool AKMR140.2 [Appendix 1](#).
- 80mL water.

4. Procedure

4.1 Ensure competency

Screening for dysphagia must only be undertaken by nursing and allied health staff that have completed training, and are deemed competent in using the RBWH DST (AKMR140.2) as assessed by a Speech Pathologist (SP).

4.2 Pre-Screening Procedure

Note: Dysphagia screening is to be performed where suction equipment is available and functioning. As per [AKG suctioning Procedure](#).

- Ensure patient is alert and cooperative.
- Patient identification and procedure matching processes are undertaken according to [EMHS Patient Identification Policy](#).
- Screening procedure is explained to patient and family/caregiver, and appropriate consent is given. [EMHS Consent Policy](#).
- Provide the opportunity for an accredited interpreter where appropriate to the patient's language or communication requirements – refer to the [AKG Interpreter and Translator Guideline](#).
- Patient privacy and dignity is maintained and distractions are removed.
- Ensure patient is seated in an upright position, as clinically indicated.
- Assess patient for mouth care and undertake as required.

- Hand hygiene must be carried out as per [AKG Hand Hygiene Procedure](#).

4.3 Post-screen requirements

Following administration of the DST, clinicians are to follow the management recommendations, described on the tool as follows:

Patients identified as being at risk of dysphagia/aspiration:

- Refer to the SP for dysphagia assessment by the usual procedure (i.e. document in the medical record, and referral via patient journey board and/or contact SP directly).
- Place NBM in consultation with the medical team.
- Consider intravenous fluids / nasogastric feeding in consultation with the medical team. See the [After-hours enteral feeding regime](#).
- Consider referral to dietitian to ensure nutrition and hydration requirements remain optimal.

Patients who are not identified as being dysphagic or an aspiration risk:

- Place on their usual diet and fluids (ensure records/handovers are checked to ensure latest diet and fluids recommendations).

In addition to this consider the following requirements:

- Patient to remain in an upright position for a minimum of 30 minutes.
- Notify shift coordinator and/ or senior clinician re: outcome of screen.
- Inform catering staff of any dietary changes.
- Update iSoft Clinical Manager (iCM) – Nursing Handover ‘diet/diet notes’.
- Clinical staff can monitor the swallow of all patients who have commenced oral intake using the RBWH Daily Swallow Screen [Appendix2](#) as required.

5. Roles and responsibilities

Role	Responsibility
Speech Pathology (SP) Department	• SP Coordinator requests attendance list 1 week prior to scheduled session from Education & Professional Development (EPD). • Promote attendance as required. Minimum of three (3) staff to attend each training session for education to be undertaken. • Print attendance sheet and take to education sessions for staff to sign. • Conduct Dysphagia Screening Tool (DST) presentation. • Determine if staff deemed competent, determined by achieving minimum 80% and 5/5 weighted questions answered correctly. • Records competency on attendance sheet with a Y (yes) or N (no) in column HRIS Skill: DYSWA, Code: 001. • Scans attendance sheet and emails to AHS.EPD@health.wa.gov.au • Provides certificate of completion to Allied Health staff.

Role	Responsibility
	- Identify incorrect screening and escalate to the Nurse Unit Manager (NUM) or Associate Nurse Unit Manager (ANUM). - SP coordinator to request quarterly report from EPD to be sent to SDN and AH management for tabling at relevant committee meetings.
Nursing (NUM/ANUM/SDN) and Allied Health Management	- Book in nursing/allied health staff for DST training sessions. - NUM to liaise with Speech Pathologist and Staff Development Nurse (SDN) to organise education and training. - If any concerns with correct completion of DST screen are identified, a request to Speech Pathology to review the nursing staff member's ability to accurately complete assessment must be promptly made. - It is the responsibility of the individual nurse to maintain their own competency through yearly education via the eLearning located on the EPD Information Hub page. SDNs will advise individual staff when annual refresher eLearning is due following receipt of HRIS reports from EPD.
Education and Professional Development	- Creates monthly sessions on EPD calendar. - Books staff in as notified. - Upon receipt of DST attendance sheet update staff record in HRIS. - Upon receipt of eLearning certificate, update HRIS record with HRIS Skill: AP162, Code: NA. - Pull quarterly report from HRIS and send to SDN and Allied Health Advisory Committee to be tabled (as prompted by SP coordinator on the first Monday of the months of Mar, Jun, Sept & Dec).

6. Legislative context

The following documents are required to give effect to this procedure:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP0095](#)
- East Metropolitan Health Service (EMHS) [Consent Policy EMHS: 5](#)
- EMHS [Patient Identification and Procedure Matching Policy EMHS: 21](#)

7. Supporting information

The following documents support the implementation of this procedure:

- [Interpreter and Translator Guideline AKG-GUI-0212-00](#)
- [Suctioning – Airway AKG-PRO-0430-11](#)
- [Hand Hygiene AKG-POL-0187-17](#)

8. Compliance, monitoring and evaluation

Compliance against this procedure will be monitored and evaluated by the Allied Health Advisory Committee.

Monitoring activities will include:

- At a minimum annual audit of correctly completed DST forms
- Speech Pathologists to keep a record of patients who are NBM pending SP review where a DST was not completed (and its use could have prevented the patient being NBM)
- Monitoring of reported clinical incident data via Datix CIMS
- % of staff trained/competent in use of DST
- % of staff compliant/up to date with completion of annual eLearning refresher
- Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.20: The health service organisation uses its training systems to:
- Standard 1 – Action 1.27: The health service organisation has processes that:
- Standard 2 – Action 2.4: The health service organisation ensures that its informed consent processes comply with legislation and best practice
- Standard 5 – Action 5.10: Clinicians use relevant screening processes:
- Standard 5 – Action 5.11: Clinicians comprehensively assess the conditions and risks identified through the screening process
- Standard 5 – Action 5.12: Clinicians document the findings of the screening and clinical assessment processes, including any relevant alerts, in the healthcare record
- Standard 5 – Action 5.27: The health service organisation that admits patients overnight has systems for the preparation and distribution of food and fluids that include nutrition care plans based on current evidence and best practice
- Standard 5 – Action 5.28: The workforce uses the systems for preparation and distribution of food and fluids to:
- Standard 6 – Action 6.3: Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers and families during high-risk situations to:

10. References

1. Stroke Foundation. (2017). *Clinical guidelines for stroke management 2017*. Melbourne, Australia. Retrieved from <https://informme.org.au/Guidelines/Clinical%20Guidelines%20for%20Stroke%20Management%202017>
2. Royal Brisbane Womens Hospital Dysphagia Screening Tool (RBWH DST), 2006
Queensland Health, Speech Pathology Department, Herston 4029
3. SMHS Clinical Practice Standards, SMHS CPS Code: CPSP003

11. Glossary

The following definitions are relevant to this policy.

Term	Definition
Dysphagia	Dysphagia is the term used to describe difficulty swallowing food or drink at the oral, pharyngeal and/or oesophageal stage.
RBWH Dysphagia Screening Tool (DST)	A dysphagia screening tool developed by the Royal Brisbane Woman's Hospital (RBWH) designed to guide clinical staff in the identification and management of patients at risk of dysphagia /aspiration. The tool is deemed statistically valid and reliable on the use in general acute medical and surgical populations as well as with acute stroke populations.

12. Policy contact

Enquiries relating to this procedure may be directed to:

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 Department: Speech Pathology
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13. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	07/07/2023	07/07/2026	Minor formatting/punctuation amendments

14. Authorisation

This procedure has been authorised and issued by the Director Allied Health.

Authorised by	Director of Allied Health and Ambulatory Care
Authorisation date	06/07/2023
Published date	07/07/2023

Appendix 1: RBWH Dysphagia Screening Tool

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ARMADALE HEALTH SERVICE		Please use I.D. Label or BLOCK PRINT	
RBWH DYSPHAGIA SCREENING TOOL		Family Name:	UMRN:
WARD _____		Given Names:	
DOCTOR _____		Gender:	DOB:

Screening Procedure for Patients with Dysphagia and / or Aspiration Risk Only to be used by trained clinical staff	
PHASE 1	
Does the patient have any of the following or previous history of the following? (tick if applicable)	
☐ Yes / ☐ No	Dysphagia or aspiration risk on previous admission(s)
☐ Yes / ☐ No	COPD +/- upper GI disorder
☐ Yes / ☐ No	Stroke
☐ Yes / ☐ No	Neurological involvement
☐ Yes / ☐ No	Head injury
☐ Yes / ☐ No	Head & neck (H&N) cancer +/- H&N surgery / chemotherapy / radiation
☐ Yes / ☐ No	Orthopaedic conditions: # NOF, cervical spinal surgery
☐ Yes / ☐ No	Acutely unwell AND frail aged with co-morbidities
☐ Yes / ☐ No	Suspected aspiration pneumonia / recurrent chest infections
☐ Yes / ☐ No	Severe disability (e.g. physical disability)
- If 'Yes' to any of the above boxes proceed to Phase 2 - If all boxes are ticked 'No', patient to have usual modified diet or general diet & thin fluids. - Complete 'Swallowing Management Plan' below.	
PHASE 2	
Interview with Patient / Family / Carer (tick if applicable or assessable)	
☐ Yes / ☐ No	Do you have difficulty eating food / drinking fluid?
☐ Yes / ☐ No	Do you cough or have choking episodes whilst eating / drinking?
☐ Yes / ☐ No	Do you need to modify the food you eat? e.g. softer or smaller pieces
If yes, provide details re current diet / fluid consistency: Diet _____ Fluid _____	
☐ Yes / ☐ No	Do you get chest infections? Is this recent? ☐ Yes / ☐ No
Has it been investigated before? ☐ Yes / ☐ No	
Indicators of Dysphagia / Aspiration Risk (tick if applicable)	
☐ Yes / ☐ No	Altered LOC / reduced responsiveness
☐ Yes / ☐ No	Respiratory rate >30bpm
☐ Yes / ☐ No	Slurred speech (dysarthria)
☐ Yes / ☐ No	Weak / absent volitional cough
☐ Yes / ☐ No	Not managing oral secretions (drooling, wet voice)
☐ Yes / ☐ No	Weak voice (dysphonia)
☐ Yes / ☐ No	Suspected aspiration pneumonia / recurrent chest infections
☐ Yes / ☐ No	Documented or history of dysphagia / aspiration risk
☐ Yes / ☐ No	Reports difficulty swallowing
☐ Yes / ☐ No	Reported coughing / choking episodes whilst eating / drinking
Water Test: Administer only if 'no' has been ticked for all indicators of dysphagia. Give the patient 85mls (1/3 cup) water	
☐ Yes / ☐ No	Coughing during or between swallows, or up to one minute after swallowing
☐ Yes / ☐ No	Wet / gurgly or hoarse voice after swallowing
☐ Yes / ☐ No	Increased respiratory rate after swallowing
- If 'Yes' to any of the above Indicators or the Water Test , refer patient to speech pathologist. Patient should remain NBM until speech pathologist review in consultation with the medical team. If a speech pathologist is unavailable, consider nutrition / hydration options such as giving IV fluids / NG feeds under medical supervision/advice. - If 'No' to all of the above Indicators and the Water Test , place patient on usual modified diet or general diet & thin fluids	
SWALLOWING MANAGEMENT PLAN	
☐ Yes / ☐ No	Patient identified as dysphagic or aspiration risk
If 'Yes': Recommended patient remain NBM ☐ Yes / ☐ No IV fluids ☐ Yes / ☐ No NG feeds ☐ Yes / ☐ No	
Medical officer notified ☐ Yes / ☐ No Date referred: ____/____/____	
Speech pathologist notified ☐ Yes / ☐ No Date referred: ____/____/____	
If 'No': ☐ Patient placed on general diet and thin fluids; or	
☐ Patient placed on usual modified diet / fluids: Diet _____ Fluids _____	
Print Name _____ Signature _____ Designation _____	
Ward _____ Unit _____ Date _____ Time _____	

AKMR140.2 RBWH DYSPHAGIA SCREENING TOOL

Appendix 2: RBWH – Daily Swallow Screening

